Fax cover sheet

Harborview Coast Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Renata Solberg, RN CM
Phone: 555-0171
Date: 10/14/2026 10:05
Pages (including cover): 12
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

47M pneumonia, cough improving. Ambulating independently, self-care.
Requesting recuperative care placement 10/15. Please call CM with questions.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH • HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Harborview Coast Hospital (fictional)
Dept/Floor: 5 West Medical
SW/CM/RN: Renata Solberg, RN CM
Phone/Pager #: 555-0171
Email: rsolberg@harborview.example
Nursing Station #: 555-0172
Authorized by: Dr. L. Abernathy
Phone #:

Patient Information

Patient Name: Marcus Doe MRN: HC-4418207 Date of Birth (dd/mm/yyy): 07/22/1979
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: Hispanic or Latino
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English, Spanish
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Pneumonia, right upper lobe; weight loss
Admit Date: 10/09/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 10/09/2026 Results: Chest X-ray negative

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* ☒ No	Active Infection	☒ Yes* No
*Explain		*Explain pneumonia	
Self Administer All Meds	☒ Yes No*	Ambulatory	☒ Yes No
Continent of Bowel & Bladder	☒ Yes No	Assistive Device	Yes* ☒ No
Colostomy/Ileostomy	Yes ☒ No	*Explain	
Foley Catheter	Yes ☒ No	Hospice	Yes* ☒ No
Diabetes	Yes ☒ No	*Explain	
Insulin	Yes ☒ No	Car	Yes ☒ No
Diabetes related Oral Meds	Yes ☒ No	Service Animal	Yes ☒ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ☒ No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain None

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Marcus Doe

DOB: 07/22/1979

Member ID: OCH-55730412

Referring Provider

Facility: Harborview Coast Hospital (fictional)

Clinical

Primary Diagnosis: Pneumonia, unspecified organism (RUL)

ICD-10: J18.9

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 10/15/2026

Requested Days: 14

Admit Date: 10/09/2026

Clinical summary: RUL pneumonia, room air, independent ADLs.

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Harborview Coast Hospital (fictional) - PATIENT FACE SHEET

Patient: Marcus Doe
MRN: HC-4418207
Account #: 99418207
DOB: 07/22/1979
Sex: Male
Preferred Language: English
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/09/2026 16:20 (ED arrival 10/09/2026 11:55)
Admit Type: Inpatient
Unit/Bed: 5W-512 (negative pressure room)
Attending: Lorraine Abernathy, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical

HISTORY AND PHYSICAL

Date: 10/09/2026

CC

Cough x 6 weeks, night sweats, weight loss

HPI

47 y/o M, unhoused x 4 yrs (stays in congregate shelters and outdoors), presents with productive cough x 6 weeks, intermittent blood-streaked sputum, drenching night sweats and about 15 lb unintentional weight loss. Reports a shelter contact with a "TB scare" earlier this year. No prior TB testing on file.

PMH

- Tobacco use
- No known HIV testing on file

Meds PTA

None

Allergies

NKDA

Social

Tobacco 1/2 ppd. EtOH socially, last drink 2 weeks ago. Denies drug use.

Exam

T 100.6F HR 98 BP 126/80 RR 18 SpO2 96% RA. Coarse breath sounds RUL. Thin. AOx4.

Assessment/Plan

1. RUL cavitary lesion with chronic cough, night sweats, weight loss - suspected pulmonary tuberculosis. Airborne isolation in negative pressure room. Sputum AFB smear and culture x3, TB PCR. HIV, QuantiFERON. Infectious Disease consult.
2. Tobacco use - nicotine patch
3. Dispo - SW/CM consult (unhoused). No discharge until ID clears isolation.

Signed: Lorraine Abernathy, MD 10/09/2026 18:30

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#5

Date: 10/14/2026 07:50

S: Cough persists, less sputum. Night sweats improving. Eating better.

O: T 99.4F HR 88 BP 122/76 RR 16 SpO2 96% RA. Coarse RUL breath sounds. AOx4.

A/P:

1. Suspected pulmonary TB, RUL cavitary lesion - remains on AIRBORNE ISOLATION. Sputum AFB smear #1 negative, smears #2 and #3 pending. TB PCR pending. ID following; ID has NOT cleared isolation.
2. Empiric RIPE therapy started 10/12 per ID (rifampin, isoniazid, pyrazinamide, ethambutol) with pyridoxine.
3. Dispo - not medically ready for congregate setting until isolation cleared by ID and public health. CM aware.

Signed: Lorraine Abernathy, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Infectious disease consult note

INFECTIOUS DISEASE CONSULT - FOLLOW-UP

Date: 10/13/2026 15:10

Imaging reviewed: RUL thick-walled cavitary lesion with surrounding nodular opacities, highly concerning for active pulmonary TB.

Sputum AFB smear #1 negative 10/11. Smears #2, #3 and TB PCR pending. Cultures take up to 6-8 weeks.

Recommendations:

- Continue airborne isolation. NOT cleared for discharge to congregate or shared living setting.
- Isolation may be reconsidered only after 3 negative AFB smears and negative TB PCR, or after adequate treatment with clinical response, per ID and County Public Health.
- Continue RIPE + pyridoxine; directly observed therapy (DOT) referral to County TB clinic.

Signed: N. Vasquez-Hale, MD (Infectious Disease)

-- FICTIONAL DEMO RECORD - not a real patient --

Physical therapy evaluation

PHYSICAL THERAPY EVALUATION

Date: 10/13/2026 11:15

Seen in room only (airborne isolation); hallway gait not tested.

Bed mobility: independent

Transfers: independent

Gait: in room approx 40 ft x 3 laps, independent, no assistive device

Balance: good

Assessment

No skilled PT needs. Expect independent community ambulation; distance >100 ft not tested due to isolation.

Signed: J. Whitcombe, PT, DPT

-- FICTIONAL DEMO RECORD - not a real patient --

Nursing shift note

NURSING SHIFT NOTE

Date: 10/14/2026 06:15

Pt AOx4. Independent with ADLs, showers independently in room bathroom. Continent of bowel and bladder.

Takes RIPE meds with RN direct observation; states names and purpose of each med.

Calm and cooperative. Wears surgical mask when staff in room.

Isolation status: AIRBORNE - negative pressure room, N95 required for staff.

Signed: P. Delacroix, RN

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Social work note

SOCIAL WORK NOTE

Date: 10/13/2026 14:00

Pt pleasant and engaged, AOx4. Denies SI/HI. Denies hallucinations. No psychiatric history.

No conservatorship. Not enrolled in FSP.

Unhoused x 4 yrs; stays in congregate shelters. Public health notified of suspected TB for contact investigation.

Signed: Renata Solberg, RN CM / E. Thorsen, MSW

-- FICTIONAL DEMO RECORD - not a real patient --

Labs, imaging and vitals

LABS

10/09 | WBC 10.4 | Hgb 12.1 L | Plt 388 | Na 135 | K 4.1 | Cr 0.8 | Albumin 3.1 L

10/09 HIV 1/2 Ag/Ab: Nonreactive

10/10 QuantiFERON-TB Gold: Positive

10/11 Sputum AFB smear #1: Negative (no AFB seen)

10/12 Sputum AFB smear #2: Pending

10/13 Sputum AFB smear #3: Pending

10/11 MTB PCR (sputum): Pending

10/09 MRSA nares PCR: Negative

10/14 05:30 SARS-CoV-2 rapid antigen: Negative

RADIOLOGY REPORT

10/09 CXR PA/lat: Right upper lobe cavitary lesion, 3.2 cm, with surrounding nodular opacities. No effusion. No pneumothorax.

Impression: Findings concerning for active pulmonary tuberculosis. Recommend airborne isolation.

10/10 CT chest: RUL thick-walled cavity with tree-in-bud nodularity. No pulmonary embolism.

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/13 08:00	99.1	90	124/78	16	96%	RA
10/13 14:00	99.6	92	126/80	18	96%	RA
10/13 20:00	99.2	88	120/74	16	95%	RA
10/14 02:00	98.8	84	118/72	16	96%	RA
10/14 07:50	99.4	88	122/76	16	96%	RA

-- FICTIONAL DEMO RECORD - not a real patient --

Current medication list

CURRENT MEDICATIONS

Patient: Marcus Doe MRN: HC-4418207

- Rifampin 600 mg PO daily (DOT)
- Isoniazid 300 mg PO daily (DOT)
- Pyrazinamide 1500 mg PO daily (DOT)
- Ethambutol 1200 mg PO daily (DOT)
- Pyridoxine 50 mg PO daily
- Nicotine patch 14 mg/24h daily

No IV medications. No home O2.

Discharge medication list: not yet prepared

-- FICTIONAL DEMO RECORD - not a real patient --